

ADA Master Table — Companion Evidence Report

Generated: 2026-04-03 07:27
Panel size: 136 antibodies
File: data/ada_master_136_curated.csv

1. ADA Evidence Tier Breakdown

- **Tier A:** 94 (69%)
 - **Tier B:** 36 (26%)
 - **Tier C:** 6 (4%)
- Tier definitions:**
- *Tier A:* ADA value anchored by PMID, FDA label, or ClinicalTrials.gov. Evidence chain from primary source.
 - *Tier B:* Real URL verified; ADA value confirmed by automated text matching. Evidence narrative may be AI-paraphrased (not verbatim).
 - *Tier C:* Unverified or known data quality issues. Excluded from CLEAN-129 analysis.

2. Column Coverage Summary

Column Group	Representative Field	Present	Coverage
Identity	antibody_name	136/136	100%
Target & Indication	indication_text	136/136	100%
Disease Class	disease_class_curated	136/136	100%
Fc Engineering	fc_engineering	136/136	100%
Fc Effector Status	fc_effector_status	136/136	100%
Fc Mutation Notes	fc_mutation_notes	136/136	100%
Route	route_curated	136/136	100%
Dose (mg)	dose_mg	136/136	100%
Dose Frequency	dose_freq	136/136	100%
Half-life (days)	half_life_days	136/136	100%
Assay Platform	assay_platform	136/136	100%
Assay Generation	assay_generation	136/136	100%
Clinical ADA (%)	ada_first_pct	135/136	99%
ADA Evidence Chain	ada_evidence_chain_excerpt	136/136	100%

Column Group	Representative Field	Present	Coverage
ADA Source URL	<code>ada_source_url_primary</code>	130/136	96%
ADA Evidence Tier	<code>evidence_tier</code>	136/136	100%
VH Sequence	<code>vh_seq</code>	131/136	96%
VL Sequence	<code>vl_seq</code>	131/136	96%
VH Germline Identity	<code>vh_germline_identity</code>	131/136	96%
VL Germline Identity	<code>vl_germline_identity</code>	131/136	96%
PDB Structure	<code>pdb_path</code>	131/136	96%
CMC pI	<code>pI</code>	131/136	96%
MHC-II TCIA Score	<code>immuno_tcia_score</code>	131/136	96%
SASA Surface Patches	<code>surf_n_patches</code>	131/136	96%
V2 Model Score	<code>ada_v2_score</code>	131/136	96%
Confounder Source	<code>confounder_source</code>	136/136	100%

3. Source Provenance

Source	File	Fields Contributed
136-panel spine	<code>data/immunogenicity_panel_136_master.csv</code>	Sequence, germline, CMC, MHC-II, SASA, structure, V2 model scores
ADA Clinical DB	<code>data/ADA_reliable_package/clinical_db/</code>	<code>ada_evidence_chain_excerpt</code> , <code>ada_source_url_primary</code> , <code>ada_source_pmids</code> , evidence tier
Curated Confounders (70)	<code>data/reference/clinical_confounders_70.csv</code>	<code>dose_mg</code> , <code>dose_freq</code> , <code>route_curated</code> , <code>fc_engineering</code> , <code>fc_effector_status</code> , <code>half_life_days</code> , <code>assay_*</code> , <code>mtx_comedication</code> , <code>immunosuppressant_context</code>
Route & Context	<code>data/reference/route_and_context.csv</code>	<code>route_curated</code> (where not in 70), <code>oncology_indication</code> , <code>checkpoint_inhibitor</code> , <code>immune_depleting</code>

Source	File	Fields Contributed
Curated 66 Metadata	<code>data/curated_66_clinical_metadata.csv</code>	<code>indication_text</code> , <code>disease_class_curated</code> , <code>route_curated</code> , <code>dose_mg</code> , <code>dose_freq</code> , <code>half_life_days</code> , <code>fc_engineering</code> , <code>fc_effector_status</code> , <code>fc_mutation_notes</code> , <code>assay_*</code> for antibodies not in 70-panel
P70 Indications	Script-curated from FDA labels	<code>indication_text</code> , <code>disease_class_curated</code> for 70- panel antibodies

4. Known Gaps & Caveats

Missing Primary ADA Source URLs (6 antibodies)

- Naxitamab
- Retifanlimab
- Etesevimab
- Relatlimab
- Olokizumab
- Rozanolixizumab

Missing Numeric ADA Value (1 antibodies)

- Bezlotoxumab

Missing VH Sequence (5 antibodies)

- Emicizumab
- Faricimab
- Ozoralizumab
- Tarlatamab
- Zenocutuzumab

Tier C Entries (Recommended Exclusion from Quantitative Analysis)

- Naxitamab (reported: 8%)
- Retifanlimab (reported: 2.8%)
- Etesevimab (reported: 1.7%)
- Relatlimab (reported: <2%)
- Olokizumab (reported: 10-15%)
- Rozanolixizumab (reported: 10-15%)

5. Model-Derived vs Curated Fields

CURATED (evidence-backed):

- All `ada_*` fields (clinical ADA value, evidence chain, source URL, tier)
- `route_curated`, `dose_mg`, `dose_freq`, `half_life_days`
- `fc_engineering`, `fc_effector_status`, `fc_mutation_notes`
- `indication_text`, `disease_class_curated`
- `assay_platform`, `assay_generation`

MODEL-DERIVED (computational predictions, not clinical facts):

- `ada_v2_score`, `ada_v2_risk` — V2 scorer output, Spearman rho = 0.19 vs clinical ADA
- `ada_profile_disease`, `ada_profile_route`, `ada_profile_half_life` — inferred metadata for V2 auto-routing
- `immuno_tcia_score`, `immuno_risk_level`, `immuno_n_*` — MHC-II epitope predictions
- `surf_*` — SASA-based surface immunogenicity features

NEVER conflate model-derived immunogenicity scores with curated clinical ADA incidence.

6. Interpretation Guidelines

1. **Clinical ADA rates are not directly comparable across antibodies** due to differences in assay platforms, cut-points, follow-up durations, and patient populations.
2. **Tier A evidence** is the most reliable; Tier B is confirmed but paraphrased; Tier C should be treated as unverified.
3. **V2 model predictions** are relative risk rankings, not absolute ADA rate estimates. Use for within-project candidate prioritization.
4. **Subgroup analysis** (natural vs engineered) is essential — the two groups have different ADA mechanisms and confounder profiles.
5. For full discussion of confounders and limitations, see [docs/ADA_Review_Discussion_Notes.md](#).

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